TARGET AUDIENCE: ADVANCED PHYSIOLOGY INITIATES.
MISSION DIRECTIVE: DECONSTRUCT CHEMICAL DEPENDENCE, WITHDRAWAL THERMODYNAMICS, & AMINOGLYCOSIDE PROTOCOLS.
THE ALCHEMIST'S MAINFRAME
LECTURE II: ADDICTION & TOXICOLOGY

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The Denial Circuit

CORE MALFUNCTION: DENIAL

DEFINITION: Refusal to accept the reality of the systemic problem.

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ABUSE PROTOCOL

ACTION: CONFRONT.

Detail: Point out the discrepancy. "You say [X], but you do [Y]."

Warning: Use 'I' statements. Avoid 'YOU' (Aggression).

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LOSS/GRIEF PROTOCOL

ACTION: SUPPORT.

Detail: Denial protects the psyche from loss. Allow it.

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Symbiotic Pathology

Abuser

Significant Other

DEPENDENCY

- Abuser forces S.O. to make decisions.
- Abuser gets a free ride.

CODEPENDENCY

- S.O. derives self-esteem from compliance.
- S.O. feels 'good' about paying the price.

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INTERVENTION PROTOCOL:

1. SET LIMITS.
2. ENFORCE LIMITS.
3. REPAIR SELF-ESTEEM (Codependent Only).

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Manipulation: The Override

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MANIPULATION:

Abuser coerces S.O. into acts that are NOT in their best interest.

DEPENDENCY vs. MANIPULATION ⟶ DIFFERENTIAL DIAGNOSIS = HARM.

If the act is Dangerous, Harmful, or Illegal → MANIPULATION.

Example: 49yo Mother asks 17yo Son to buy alcohol (Illegal/Harmful).

PROTOCOL: SET LIMITS & ENFORCE.

(No self-esteem work required).

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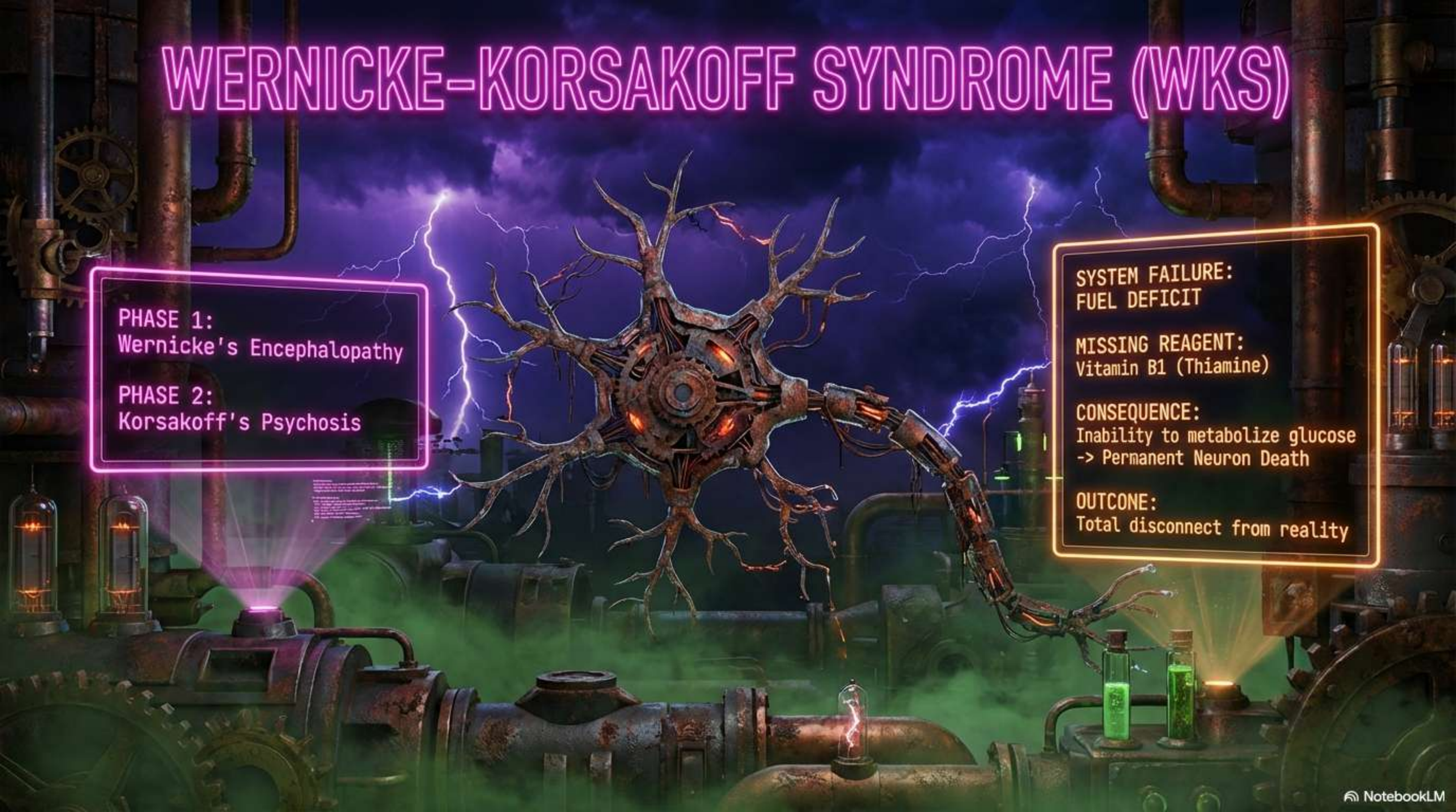
WERNICKE-KORSAKOFF SYNDROME (WKS)



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SYMPTOM: CONFABULATION

1. AMNESIA (Memory Loss).
2. CONFABULATION (Fabrication of Reality).

CRITICAL NOTE: The patient BELIEVES the lie. To them, it is absolute reality.

Operating Protocol:
INTERVENTION:
STOP: Do Not Confront. Do Not Present Reality.
GO: REDIRECT.
Technique: Channel patient from the impossible (e.g., meeting Obama) to the doable (e.g., watching TV).

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THE GENESIS GEAR: VITAMIN B1

Prognosis Data

1. **PREVENTABLE.**
   Action: Administer B1.
2. **ARRESTABLE.**
   Action: Administer B1 to stop progression.
3. **IRREVERSIBLE.**
   Warning: 70% of cases involve permanent brain damage.

DIRECTIVE: You do not have to stop drinking, but you MUST take your B1.

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AVERSION THERAPY: DISULFIRAM

AGENT: Antabuse / Revia.
MECHANISM: Induces catastrophic illness if alcohol is introduced.
TIMING: 2 Weeks to Onset / 2 Weeks Duration.

PATIENT EDUCATION - AVOID ALL HIDDEN ALCOHOL:

- Mouthwash, Cologne, Aftershave
- Insect Repellent, Hand Sanitizer
- OTC Elixirs, Uncooked Icings (Vanilla Extract)
- Note: Red Wine Vinaigrette is Safe.

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THE BINARY DRUG PROTOCOL

THE 5 UPPERS

1. Caffeine
2. Cocaine
3. PCP/LSD
4. Methamphetamines
5. Adderall

THE DOWNERS

EVERYTHING ELSE. (Alcohol, Heroin, Benzos, Marijuana, etc.)

Symptom Readout

UPPER: Tachycardia, Agitation, Borborygmi, +4 Reflexes, Seizures.

DOWNER: Bradycardia, Lethargy, Respiratory Depression/Arrest.

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THERMODYNAMICS: OVERDOSE vs. WITHDRAWAL

OVERDOSE PRESSURE

THE MATRIX

1. UPPER OVERDOSE (Too much UP)
-> EVERYTHING GOES UP (Seizure Risk).

2. DOWNER OVERDOSE (Too much DOWN)
-> EVERYTHING GOES DOWN (Resp. Arrest).

3. UPPER WITHDRAWAL (Not enough UP)
-> EVERYTHING GOES DOWN (Resp. Arrest).

4. DOWNER WITHDRAWAL (Not enough DOWN)
-> EVERYTHING GOES UP (Seizure Risk).

WITHDRAWAL VACUUM

CRITICAL ALERT: Downer Withdrawal (Alcohol) mimics Upper Overdose (Cocaine).

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THE NEWBORN CHRONOMETER

SCENARIO:
Infant born to addicted mother.

THE RULE OF 24

< 24 HOURS OLD

Assumption: INTOXICATION.

Status: Maternal supply intact.

> 24 HOURS OLD

Assumption: WITHDRAWAL.

Status: Supply cut. System crashing.

EXAMPLE: Newborn (24h+) of Quaalude (Downer) addict.

Diagnosis: Downer Withdrawal.

Symptoms: Everything goes UP (Shrill cry, Seizures, Hyper-Reflexia).

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AWS (Alcohol Withdrawal Syndrome)

Time: 24h Post-Cessation.

Status: STABLE.

Protocol: Regular Diet, Semiprivate Room, Up Ad Lib.

RESTRAINTS: NONE.

DTs (Delirium Tremens)

Time: 72h Post-Cessation.

Status: UNSTABLE / LETHAL.

Protocol: NPO/Clear Liquids, Private Room (Near Station), Restricted Bed Rest.

RESTRAINTS: REQUIRED (Vest / 2-Point).

SHARED MEDS:
Anti-hypertensives, Tranquilizers, Vitamin B1.

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HEAVY ARTILLERY: AMINOGLYCOSIDES

THE MNEMONIC: 'A MEAN OLD MYCIN'.

Indication: Severe, Life-Threatening, Resistant, Gram-Negative Infections.

Identification Protocol

RULE: Must end in -MYCIN (Gentamycin, Vancomycin).

EXCEPTION: The 'Thro' Rule.

If it has 'THRO' (Azithromycin, Erythromycin) -> THROW it off the list. These are NOT heavy artillery.

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TOXICITY SENSORS

TARGET 1: OTOTOXICITY.

Assoc: Nycin = Mice (Big Ears).
Monitor: Hearing, Tinnitus, Vertigo.
Damage: Cranial Nerve 8.

TARGET 2: NEPHROTOXICITY.

Assoc: Ear is shaped like a kidney.
Monitor: CREATININE (24hr Clearance > Serum).
Note: #1 Indicator of Renal Function.

ROUTE: IV or IM (Not absorbed PO).

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THE EXCEPTION: PO ADMINISTRATION

PO ROUTE - STERILIZATION MODE

RULE: PO Mycins are NOT absorbed. They sterilize the gut.

INDICATIONS:

1. Hepatic Encephalopathy (Kills E. Coli -> Lowers Ammonia -> Saves Brain).
2. Pre-Op Bowel Surgery (Sterilizes piping).

Serrgeant's Chant
Who can sterilize my bowel? NEO KAN!
KAN! (Neomycin & Kanomycin)

ROUTE: PO (NOT ABSORBED).
LOCAL ACTION ONLY.

WARNING: PO = STERILIZATION ONLY.
NO SYSTEMIC EFFECT.

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